

GREENWOOD-SLIDELL VFD

FIRST RESPONDER EMS

Lecture slides for the Wise County First Responder protocols

Modules E1–E8 · Taskbook Phase E · 12 scenarios

WE START CARE. WISE COUNTY EMS TAKES IT.

1 · We arrive

The highest-certified GSVFD member leads patient care.

2 · We treat

Assess and treat by protocol, in priority order.

3 · We hand off

Report to the Wise County medic. Care is theirs at patient contact.

All responders: every protocol's main steps

EMT-B and above: medication assists, aspirin, oral glucose

GSVFD First Responder EMS · About 20 class hours, 27 skills hours · Not a certification course

MODULE E1

FIRST RESPONDER ROLE AND GENERAL ORDERS

Protocols II, IV, V, VI | Taskbook E.1–E.3, E.25 | 2 hr class · 1 hr skills

By the end of this module you can:

- State who leads care and when it passes to Wise County EMS
- List the 11 general orders in order
- Handle a refusal and know when the ambulance may be cancelled
- Name the air evacuation criteria and the ride-along rules

THE 11 GENERAL ORDERS, EVERY PATIENT

1. Scene safety and patient condition
2. Responsiveness
3. Airway, breathing, circulation
4. Control severe bleeding
5. Level of consciousness
6. Reassure and keep the patient informed
7. Brief history from patient, family, bystanders
8. Vitals and physical exam; medical alert tag
9. Medications and allergies
10. Treat by protocol, in priority order
11. Complete the First Responder report

REFUSALS, AIR EVACUATION AND RIDE-ALONGS

Cancelling EMS (IV)

- Adult, conscious, fully oriented, refuses
- Parent, guardian, school or daycare staff for a minor
- Healthcare power of attorney, unless the patient is alert and wants care
- Other groups the protocol lists
- Patient signs the refusal form

Air evacuation (V)

- Ground units can't reach the patient
- Care not available locally (major burns, neurosurgery)
- No ground units available
- Penetrating head, neck or chest trauma
- Obvious spinal cord or closed head injury
- Amputation that may be reattached
- Priapism

Riding in (VI)

- Only when needed
- The medic approves
- Work under the medic's direct supervision
- Seat belt on while moving
- Follow HIPAA

MODULE E2

PATIENT ASSESSMENT AND VITAL SIGNS

Protocol II · Appendices 1 and 3 | Taskbook E.4–E.7 | 3 hr class · 3 hr skills

By the end of this module you can:

- Size up a scene and run the initial assessment
- Take a SAMPLE and OPQRST history
- Do a DCAP-BTLS exam and take baseline vitals
- Score the Glasgow Coma Scale

HISTORY, EXAM AND BASELINE VITALS

SAMPLE

- **S**igns and symptoms
- **A**llergies
- **M**edications
- **P**ertinent past history
- **L**ast oral intake
- **E**vents leading up

OPQRST (pain)

- **O**nset
- **P**rovocation
- **Q**uality
- **R**adiation
- **S**everity, 0–10
- **T**ime

DCAP-BTLS

- Deformities, contusions
- Abrasions, punctures
- Burns, tenderness
- Lacerations, swelling
- Head to toe, call findings aloud
- Check for a medical alert tag

Baseline vitals

- Respirations 12–20/min
- Pulse 60–100/min
- Systolic BP about 90–140
- SpO_2 95% or higher
- Skin warm, pink, dry; pupils
- Repeat: 3–5 min unstable, 10 min stable

GLASGOW COMA SCALE (APPENDIX 1)

SCORE	EYES	VERBAL	MOTOR
6			Obeys commands
5		Oriented	Localizes pain
4	Open on their own	Confused	Withdraws from pain
3	Open to voice	Inappropriate words	Flexion to pain
2	Open to pain	Incomprehensible sounds	Extension to pain
1	None	None	None

Scoring

- Add the three columns
- 15 is best, 3 is worst
- Brisk sternal rub is the pain stimulus
- Note conditions that prevent a full score

MODULE E3

AIRWAY, OXYGEN AND CPR-AED

Protocol 10 · airway steps in every protocol | Taskbook E.8–E.10 | 2 hr class · 4 hr skills

By the end of this module you can:

- Open an airway with head-tilt chin-lift or modified jaw thrust
- Suction and use the recovery position
- Give oxygen by NRB and ventilate with a BVM
- Run CPR with an AED on adults and children

AIRWAY, OXYGEN AND THE AED SEQUENCE

Airway

- Head-tilt chin-lift for medical patients
- Modified jaw thrust if spinal injury is possible
- Suction; recovery position if breathing but unresponsive
- BVM with oxygen, two rescuers, watch chest rise

Oxygen

- NRB mask at 12–15 L/min in most protocols
- 10–12 L/min in shock, heat stroke, unconscious diabetic
- Cylinder safety: secure it, no open flame
- Don't over-ventilate with the BVM

CPR-AED (10)

- Only if unresponsive, pulseless and not breathing
- CPR until the AED is on
- Stop CPR, press Analyze, follow prompts
- Pulse check after analysis or signs of life
- Pediatric pads: under 8 yr or under 90 lb
- Continue until EMS takes over; rotate

MODULE E4

MEDICAL EMERGENCIES

Protocols 1, 4, 7, 11, 12, 21 | Taskbook E.18–E.20 | 3 hr class · 2 hr skills

By the end of this module you can:

- Recognize allergic reaction, breathing trouble, chest pain, stroke, diabetic emergencies and seizures
- Carry out each protocol's steps in order
- Know which medication assists are EMT-B and above only
- Reassess at the protocol's vital sign interval

MEDICAL PROTOCOLS AT A GLANCE

PROTOCOL	ALL FIRST RESPONDERS	EMT-B AND ABOVE ALSO	VITALS
1 Allergic	Airway; BVM if needed; NRB 12–15; treat for shock	Patient's own epinephrine auto-injector	3–5 min
4 Breathing	Airway; BVM if needed; sit up; NRB 12–15	Patient's own inhaler or nebulizer; check expiration	3–5 min
7 Chest pain	Treat as heart attack; position of comfort; calm; NRB 12–15; OPQRST	Patient's nitro unless SBP under 100, max 3 doses; 2 × 81 mg chewable aspirin	3–5 min
11 Stroke / TIA	Ready to suction; NRB 12–15; neuro exam; semi-Fowler's; unconscious: lateral recumbent	—	3–5 min
12 Diabetic	Alert with gag reflex: sugar drink. Unconscious: contact EMS, suction, recovery position, NRB 10–12	Oral glucose if alert and able to swallow	5–10 min
21 Seizures	Move to floor; protect; never restrain or put anything in the mouth; then assess, NRB 12–15	—	5 min

MODULE E5

ENVIRONMENTAL, POISONING AND BEHAVIORAL

Protocols 9, 13, 17, 19, 20, 24 | Taskbook E.21–E.23 | 3 hr class · 1 hr skills

By the end of this module you can:

- Tell heat exhaustion from heat stroke and treat each
- Manage frostbite and hypothermia, including the 30-minute rule
- Handle ingested, inhaled and injected poisoning and snakebite
- Stay safe at radiation and behavioral calls

HEAT, COLD, POISONS, SNAKES AND SCENE SAFETY

Heat (17)

- **Exhaustion**, cool clammy skin: cool place, supine, vitals 10 min
- **Stroke**, over 104°F, hot dry skin, altered: contact EMS, ice packs to head, neck, armpits, groin (or wet sheet), stop if shivering, NRB 10–12, vitals 5 min

Cold (9)

- **Frostbite**: remove clothing, rewarm in 100–105°F water until numbness resolves, dry dressing, elevate, immobilize
- **Hypothermia**: contact EMS, wet clothes off, dry blankets, warm room; EMS under 30 min, don't rewarm; over 30 min, rewarming blanket or hot packs

Poisoning (19)

- **Ingested**: what, when, how much; bring container and vomit; on side
- **Inhaled**: remove from atmosphere only if safe; NRB 12–15
- **Injected**: keep quiet, remove jewelry, immobilize, watch for anaphylaxis

Snakebite (24)

- Still and quiet; remove jewelry
- Limb at or below heart level; immobilize
- **No tourniquet. No ice.**
- Photo of the snake from a safe distance

Radiation (20)

- Contact EMS; use facility experts
- Don't enter; keep bystanders out
- Serious injury: treat life threats, remove from hazard, strip outer clothing, wash skin

Behavioral (13)

- Scene safety; law enforcement for anyone dangerous or armed
- Direct, honest, continuous contact
- Avoid restraint; once on, leave it on

MODULE E6

BLEEDING, SHOCK, SOFT TISSUE AND BURNS

Protocols 2, 3, 5, 6, 14, 23 · Appendix 2 | Taskbook E.11–E.15 | 3 hr class · 3 hr skills

By the end of this module you can:

- Control bleeding, with and without an impaled object
- Treat hemorrhagic and cardiogenic shock
- Package an amputated part and dress an evisceration and chest wounds
- Classify and treat thermal, chemical and electrical burns

BLEEDING, SHOCK AND OPEN WOUNDS

Bleeding (3)

- Direct pressure and elevate
- Still bleeding: BP cuff or tourniquet above the wound, lowest pressure that stops it
- Impaled object: pressure around it, stabilize in place; cheek object out only for airway
- Vitals 5 min (10 with impaled object)

Shock (23)

- **Hemorrhagic**: stop bleeding, NRB 10–12, splint, legs up, blankets under and over
- **Cardiogenic**: NRB 10–12, semi-Fowler's; legs up only if hypotensive
- Vitals every 5 min

Amputation (2)

- Bleeding control, treat shock
- Rinse part briefly in saline; don't scrub
- Saline-moist gauze, sealed bag
- Cool container, **not on ice**
- Part goes with the patient

Evisceration (14)

- Moist sterile dressing, then occlusive cover
- Never push organs back in
- NRB 12–15; vitals every 5 min

Chest injuries (6)

- **Ribs**: NRB 12–15
- **Flail**: bulky pad and cravats, no tape; injured side down if tolerated
- **Sucking wound**: occlusive dressing taped 3 sides; uncover if breathing worsens

BURN SEVERITY AND THE RULE OF NINES

SEVERITY	3RD DEGREE	2ND DEGREE	1ST DEGREE
Minor	Under 2% (not face, hands, feet, groin, joints)	Under 15%	Under 20%
Moderate	2–10% (not those areas)	15–30%	20–75%
Critical	Over 10%, or any 2nd/3rd to face, hands, feet, groin, joints, or with airway or other injuries	Over 30%	Over 75%

Rule of Nines (adult)

- Head 9% · each arm 9%
- Chest 9% · abdomen 9%
- Upper back 9% · lower back 9%
- Each leg 18% · groin 1%
- Patient's palm ≈ 1%

Thermal and chemical

- Stop the burning; NRB 12–15
- Jewelry and loose clothing off
- Dry sterile dressings; keep warm
- Chemical: brush off dry lime, then flush 20+ min, no antidotes

Electrical

- Power off before you touch
- Look for entry and exit burns
- Expect arrest, arrhythmia, fractures, seizures
- Dry dressings; treat for shock

MODULE E7

FRACTURES, SPINE AND MULTIPLE TRAUMA

Protocols 15, 16, 18 | Taskbook E.16–E.17 | 2 hr class · 3 hr skills

By the end of this module you can:

- Check pulse, movement and sensation below an injury
- Splint as found, or with gentle traction if there's no pulse
- Hold manual stabilization and immobilize to a long board
- Work the multiple trauma steps in order

SPLINT, STABILIZE, THEN WORK THE TRAUMA ORDER

Fractures (15)

- Control bleeding
- Check pulse and sensation below
- Pulse present: splint as found
- No pulse: gentle traction while repositioning until it returns, then splint
- Joint above and below; recheck after
- Vitals 10 min; shock if needed

Head, neck, spine (16)

- Contact EMS early; manual stabilization
- Modified jaw thrust, never hyperextend
- Not breathing: BVM with oxygen per protocol
- No pressure on ear bleeding or a depressed skull
- Neuro exam before and after
- Long board; vomiting: roll as a unit

Multiple trauma (18)

- 1 · Modified jaw thrust
- 2 · Ventilate if not breathing
- 3 · Find and seal sucking chest wounds
- 4 · Control severe bleeding
- 5 · Hold the head until immobilized
- 6 · Brief exam · 7 · Treat for shock

MODULE E8

CHILDBIRTH AND SENSITIVE PATIENTS

Protocols 8, 22, 25 · Appendix 3 | Taskbook E.23–E.24 | 2 hr class · 2 hr skills

By the end of this module you can:

- Recognize imminent delivery and assist a normal birth
- Clamp and cut the cord, keep the newborn warm, score APGAR
- Manage the listed delivery complications
- Care for vaginal bleeding and sexual assault patients with privacy

DELIVERY, COMPLICATIONS AND APGAR

Normal delivery (8)

- Crowning = deliver on scene
- Gentle palm pressure on the head
- Suction mouth, then nose
- Gentle downward for top shoulder
- Baby level with vagina until clamped
- Clamps 7–10 in, then 2–3 in further; cut between
- Dry, wrap, on mom; fans and AC off

Complications

- **Sac intact**: tear it, clear the face
- **Cord on neck**: slip over head, or clamp twice and cut
- **Prolapsed cord**: knees-to-chest or hips up, NRB, warm moist cord, lift baby off cord and stay
- **Placenta**: massage uterus, never pull the cord, bag it

Sensitive patients

- **Vaginal bleeding (25)**: pad beneath, vitals 5 min, treat shock, save tissue
- **Sexual assault (22)**: privacy, same-sex responder if possible, no name on radio, preserve evidence: no washing, toileting or changing

APGAR	0	1	2
Activity	Absent	Arms and legs flexed	Active movement
Pulse	Absent	Below 100	Over 100
Grimace	Flaccid	Some flexion	Sneeze, cough, pull away
Appearance	Blue, pale	Body pink, extremities blue	Completely pink
Respiration	Absent	Slow, irregular	Vigorous cry

SCENARIO LAB · 8 HR · TASKBOOK E.26

TWELVE CALLS FROM OUR DISTRICT

1 Chest pain at the feed store	2 Collapse at the fellowship hall	3 Bee stings in the hay field
4 Confused at the kitchen table	5 Slurred speech at breakfast	6 Hay crew collapse in July
7 Motorcycle versus deer	8 Chainsaw on the fence line	9 Baby won't wait
10 Rattlesnake in the barn	11 Rollover, “I'm fine”	12 Brush pile flash burn

Each scenario has its own operator card on the next 12 slides, and in the Scenario Lab on the training library site.

Pass as team lead: every critical item, 8 of 10 performance items, and a clean handoff. E.26 signs after three passes: one medical, one trauma and one more of the trainee's choice.

SCENARIO 1 OF 12 · MEDICAL

CHEST PAIN AT THE FEED STORE

Dispatch. 62-year-old man with chest pain, feed store on the highway.

On arrival. Man sitting on a pallet, pale and sweating, rubbing his chest. He was loading feed 20 minutes ago.

Findings. Pressure, 7/10, spreading to the left arm. History of high blood pressure; takes lisinopril; has a nitroglycerin prescription he has never used. No allergies. GCS 15.

Vitals. BP 152/94, pulse 104, respirations 22, SpO2 95%, skin pale, cool and sweaty.

Expected actions (7). scene safety and gloves; loosen clothing, position of comfort, keep him calm and still; NRB 12–15 L/min; OPQRST and SAMPLE; vitals every 3–5 min; AED staged nearby. **EMT-B and above:** 2 chewable 81 mg aspirin after checking for allergy; assist his nitroglycerin if systolic BP is 100 or higher, up to 3 doses, after checking the expiration date and asking about erectile dysfunction drugs.

Progression. After the second nitroglycerin his BP is 96/60. The team stops nitroglycerin and moves to cardiogenic shock care (23): semi-Fowler's, legs up because he is hypotensive, blankets, vitals every 5 min.

Debrief: Why every chest pain is treated as a heart attack; the systolic BP cutoff; what goes in the handoff.

SCENARIO 2 OF 12 · MEDICAL

COLLAPSE AT THE FELLOWSHIP HALL

Dispatch. 70-year-old woman collapsed, CPR in progress, church fellowship hall.

On arrival. A bystander is doing compressions. A wall-mounted AED is unopened.

Findings. Unresponsive, not breathing, no pulse.

Expected actions (10). confirm arrest; take over CPR without a long pause; attach the AED; stop CPR and press Analyze; shock when advised with everyone clear; resume CPR right away; BVM with oxygen; rotate compressors every 2 minutes.

Progression. First analysis: shock advised. After the second analysis: no shock advised, pulse present, breathing 6 per minute. The team checks the pulse, keeps ventilating with the BVM, and monitors ABCs.

Debrief: Compression pauses, roles, the times and number of shocks for the handoff, and thanking and checking on the bystander.

SCENARIO 3 OF 12 · MEDICAL

BEE STINGS IN THE HAY FIELD

Dispatch. 34-year-old man stung by bees, having trouble breathing.

On arrival. Man sitting on a tailgate; hives on arms and neck, lips swelling, hoarse voice, wheezing. He has a prescribed epinephrine auto-injector in the truck.

Vitals. BP 86/52, pulse 122, respirations 28 and wheezy, SpO2 89%.

Expected actions (1, 19, 23). keep bees away from the crew; open and maintain airway; NRB 12–15 L/min, BVM if breathing fails; treat for shock; remove rings and watch (hands are swelling); keep him still; vitals every 3–5 min. **EMT-B and above:** confirm it is his auto-injector and not expired, then assist with it into the outer thigh.

Progression. Five minutes after epinephrine: wheezing better, BP 104/70. If the team doesn't give or assist with epinephrine, his voice gets hoarser and SpO2 falls to 84%.

Debrief: Signs that make it anaphylaxis; the five rights of medication assists; why he still goes to the hospital.

SCENARIO 4 OF 12 · MEDICAL

CONFUSED AT THE KITCHEN TABLE

Dispatch. 55-year-old woman acting confused, history of diabetes.

On arrival. Her husband says she took her insulin and skipped lunch. She is sweaty and slow to answer but follows commands and swallows on request.

Vitals. BP 132/84, pulse 110, respirations 18, SpO2 97%, skin cool and sweaty. GCS 14 (confused).

Expected actions (12). confirm she is alert enough, has a gag reflex and can drink unaided; orange juice with 2–3 tablespoons of sugar, or oral glucose (EMT-B and above); vitals every 5–10 min.

Progression (operator's choice). She becomes unresponsive before drinking. The team stops anything by mouth, contacts EMS, suctions, places her in the recovery position and gives NRB 10–12 L/min.

Debrief: When oral sugar is safe and when it is not.

SCENARIO 5 OF 12 · MEDICAL

SLURRED SPEECH AT BREAKFAST

Dispatch. 78-year-old man, wife says he is talking funny.

On arrival. Right facial droop, right arm drifts down, speech slurred. His wife says he was normal at 0700; it is now 0740.

Vitals. BP 188/102, pulse 88 and irregular, respirations 18, SpO2 96%. GCS 14.

Expected actions (11). airway, suction ready; NRB 12–15 L/min; neuro exam; semi-Fowler's; calm and reassure; vitals every 3–5 min; report the time he was last known well.

Progression. He vomits and becomes harder to rouse. The team suctions, places him lateral recumbent, and is ready to ventilate with the BVM.

Debrief: Why the last-known-well time matters for the hospital.

SCENARIO 6 OF 12 · MEDICAL

HAY CREW COLLAPSE IN JULY

Dispatch. 19-year-old man passed out in a hay field, 101°F.

On arrival. Crew moved him to the shade of a trailer. He is confused, skin hot and dry.

Vitals. BP 98/60, pulse 136, respirations 28, SpO2 96%. GCS 13.

Expected actions (17). contact EMS; move him to a cool place (an air-conditioned cab); remove heavy clothing; ice packs to head, neck, armpits and groin, or a wet sheet with airflow; stop cooling if he shivers; NRB 10–12 L/min; vitals every 5 min.

Progression. He has a 1-minute seizure. The team protects him, puts nothing in his mouth, then positions and suctions (21).

Debrief: Heat exhaustion versus heat stroke; why cooling starts before transport.

SCENARIO 7 OF 12 · TRAUMA, OB, REFUSAL, BURNS

MOTORCYCLE VERSUS DEER

Dispatch. Motorcycle crash, one rider down, farm-to-market road at night.

On arrival. Traffic is moving past. A 28-year-old man lies in the ditch, helmet on, talking. Bright red blood is spurting from a gash on his lower right leg. He says his neck hurts and his hands tingle.

Vitals. BP 104/68, pulse 118, respirations 24, SpO2 94%. GCS 14.

Expected actions (3, 16, 18, 23). block the lane and set up a protected work area (taskbook 1.16); manual spinal stabilization; airway by modified jaw thrust if needed; direct pressure, then a BP cuff or tourniquet above the wound when bleeding continues; treat for shock; neuro exam before and after immobilization; long spine board; vitals every 5 min.

Progression. BP falls to 86/50 if bleeding control is slow.

Debrief: Order of priorities in multiple trauma; whether tingling hands make this an air evacuation candidate (protocol V); traffic safety.

SCENARIO 8 OF 12 · TRAUMA, OB, REFUSAL, BURNS

CHAINSAW ON THE FENCE LINE

Dispatch. Man cut his hand with a chainsaw, ranch off a county road.

On arrival. 45-year-old man holding a towel on his left hand; his index and middle fingers are cut off at the middle knuckle and lying in the grass.

Vitals. BP 128/80, pulse 108, respirations 20, SpO2 98%.

Expected actions (2, 3, 23). shut down the saw; control bleeding; treat for shock; find both fingers; rinse briefly with saline, wrap in moist gauze, seal in a plastic bag, put the bag in a cool container (not directly on ice), and send them with him.

Progression. The team asks EMS about air evacuation; amputations that may be reattached are on the criteria list.

Debrief: Why the part is never put directly on ice; reattachment time window.

BABY WON'T WAIT

Dispatch. 24-year-old woman in labor, EMS 25 minutes out.

On arrival. Second pregnancy, 39 weeks, water broke 30 minutes ago, contractions 2 minutes apart, feels the urge to push. The baby's head is crowning.

Vitals (mother). BP 124/78, pulse 96, respirations 20.

Expected actions (8). history; open the OB kit; gentle pressure on the head; suction mouth then nose; deliver the shoulders; keep the baby level until clamped; clamps at 7–10 inches and 2–3 inches beyond, cut between; dry, warm and place on the mother; APGAR at 1 and 5 minutes; uterine massage; deliver the placenta without pulling the cord.

Progression (operator's choice). The cord is around the baby's neck; the team slips it over the head. One-minute findings: active movement, pulse 140, cries when suctioned, body pink with blue hands and feet, vigorous cry (APGAR 9).

Debrief: Keeping the newborn warm; when to reassess mother and baby.

SCENARIO 10 OF 12 · TRAUMA, OB, REFUSAL, BURNS

RATTLESNAKE IN THE BARN

Dispatch. 9-year-old girl bitten by a snake.

On arrival. The snake is under a pallet in the barn. Her mother carried her to the porch. Two puncture wounds on the left ankle; swelling spreading up the calf; she is crying and scared.

Vitals. pulse 120, respirations 26, BP 100/64.

Expected actions (24). stay away from the snake; keep her still and calm; remove her shoe, sock and ankle bracelet; keep the leg at or below heart level; immobilize; no tourniquet, no ice; vitals every 5 min; watch for anaphylaxis; a photo of the snake only if it can be taken safely.

Debrief: Children's normal vitals; talking to a scared child and parent.

SCENARIO 11 OF 12 · TRAUMA, OB, REFUSAL, BURNS

ROLLOVER, "I'M FINE"

Dispatch. Single-vehicle rollover into a bar ditch, one occupant.

On arrival. A 40-year-old man is out of the pickup and walking around. Small scrape on his forehead. He wants to go home.

Findings. Alert and oriented to person, place, time and events. GCS 15. No alcohol odor. Vitals normal.

Expected actions (II, IV). full assessment despite the refusal; explain the risks given the mechanism; confirm he is an adult who is conscious and fully oriented; complete the First Responder refusal form; cancel the responding unit per protocol IV; document.

Variant (operator's choice). He asks the same question three times and can't say what happened. He is not fully oriented, so EMS is not cancelled; the team keeps him calm and asks law enforcement to stay.

Debrief: What makes a refusal valid; protecting the patient and the department with good documentation.

SCENARIO 12 OF 12 · TRAUMA, OB, REFUSAL, BURNS

BRUSH PILE FLASH BURN

Dispatch. Man burned while burning brush.

On arrival. A 67-year-old man used diesel on a brush pile; it flashed. The pile is still burning near a fence. His forearms and hands are red and blistered, his eyebrows and beard are singed, and his voice is hoarse.

Vitals. BP 146/90, pulse 112, respirations 24, SpO2 95%.

Expected actions (5). make the fire scene safe; stop the burning process; NRB 12–15 L/min; remove his watch and rings now; classify (2nd degree, both forearms and hands, about 9% BSA by the Rule of Nines, plus the face); dry sterile dressings; keep him warm; vitals every 3–5 min.

Severity. Critical, because the hands and face are burned and the hoarse voice suggests an airway burn. Report degree, percentage and severity to EMS.

Debrief: Airway burn warning signs; why level of care for major burns is an air evacuation criterion.



START CARE. HAND IT OFF CLEAN.

- Get each Phase E task signed in your taskbook
- Keep your CPR card current
- Take a Texas DSHS ECA, EMR or EMT course for certification
- Know the protocols cold: they live on every apparatus

Greenwood-Slidell VFD · Wise County First Responder protocols